

إسم المريض رابعياً : ارزاق ابراهيم احمد عبدالجواد

المحافظة : الجيزة الحي / المدينة : البدرشين

العنوان : الجيزه
تاريخ تسجيل الإستمارة : 03-10-2026

رقم البطاقة : 27610202102802 رقم التليفون : 01025858996

العمر(تاريخ الميلاد) : 20-10-1976 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي بيمبروليزوماب ويريدي [14335] **قسم العلاج :** الاورام

التعليق على التشخيص : -----

GR 1-4
ر. د. يوسف
أستاذ
الدراسات
الاسلامية
جامعة
البحرين
صحة البيانات المرفقة
الاجتهاد

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نَفَقَات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: ارزاق ابراهيم احمد عبدالجواد National ID: 27610202102802	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

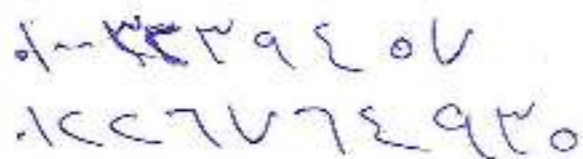
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pembrolizumab IV

التاريخ :	الوقت :
05-07-2026	-- --:--
	الطبيب المعالج : د/شيماء لاشين



07/26/2026



MDT FORM

Name:	أرزق إبراهيم أحمد عبد الجواد	MRN:	11876
Age:	50 Y	ID:	27610202102802
Menopausal Sta	POST	Phone:*	01003239457
NRS SCORE:		Marital Status:	
BMI:		Family History:	-VE
Gender			
CO Morbidity:	MED FREE	Reason Of MDT:	
			POST OP (OUTSIDE THE HOSPITAL)
Surgical History:*	TONSILLECTOMY RT EAR TEMPANOPLASTY	ECOG Performance Status	
			☐ 0 ☒ 1 ☐ 2 ☐ 3 ☐ 4

Clinical Data:

Name: ارزاق ابراهيم احمد عبدالجواد			
MR. : 11876			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 49-year-old individual who is postmenopausal. The clinical history is notable for a right-sided breast cancer previously treated with right breast-conserving surgery on 24/5/2026 performed outside Gustave. The initial pathology was invasive ductal carcinoma, grade 3, with medullary features, staged as pT1 N3 (10/15 nodes), triple-negative, Ki-67 25 %. A pre-operative PET-CT demonstrated right supraclavicular lymph node uptake with SUV 10 and a 1.8 cm lesion. Post-operative MRI of the right breast showed a BIRADS 4A non-mass-like enhancement. The patient was recommended to receive two weeks of taxol and carboplatin. A true-cut biopsy of the right breast lesion performed on 17/8/2026, corresponding to the post-operative MRI BIRADS 4 non-mass enhancement, revealed changes consistent with resection site alterations without residual disease. The biomarker profile confirmed triple-negative status with Ki-67 25 %. Imaging findings include the aforementioned post-operative MRI BIRADS 4A non-mass enhancement and the prior PET-CT supraclavicular node involvement; no additional metastatic disease was reported. The referring team recommended two weeks of taxol and carboplatin. The multidisciplinary team's outcome and plan is to continue systemic therapy and proceed with completion mastectomy after the current treatment.			
تحريراً في ,			
		التاريخ :	23-09-2026
		الوقت :	
		الطبيب المعالج :	
يعتمد ,			

BX:

IDC G3 NST WITH MEDULLARY LIKE FEATURES

ER: 0/8**PR:** 0/8**HER2:** 0**KI67:** 25**History:**

RT BREAST MASS , THEN RT WLE +AC OUTSIDE THE HOSPITAL WITHOUT BIOPSY

Sonomammography:

***THE INITIAL SM :

RT UOQ IRREGULAR ISODENSE LESION WITH CALCIFIC FOCI OVERLYING THE LESION . LESION LOCATED 20MM DEEP TO SKIN IN ZONE C AT 9 O'CLOCK 10*5*11.7 MM .MULTIPLE ENLARGED MORE THAN 3 PATHOLOGICAL LN

***POST OP MRI:

POST BCS+AC , POST OP CHANGES WITH UOQ FOCAL AREA OF NON MASS ENHANCEMENT BIRAD 4A 3*1 CM

***POST OP PET CT:

METABOLICALLY ACTIVE RT AXILLARY LEVEL 2 AND SUPRACLAVICULAR LN

Biomarker/Molecular

***PATHOLOGY REVISION:

FREE RESECTION MARGINS

AX. LN 10/15

ABSENT DCIS, ABSENT MICROCALCIFICATION

ABSENT PERINEURAL INVASION

ANGIOINVASION PRESENT

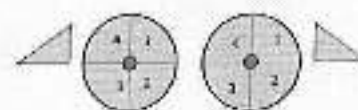
TUMOR SIZE 1*0.5CM

Mets Workup:**Recommendations:****MDT Outcome & Plan**

- 1- for trial of clip bracketing of non mass area in uoq if rejected by intervention radio so for completion mastectomy post ttt
- 2- Portcath insertion - BRCA testing
- 3- Start Paclitaxel + Carboplatin x 12 weeks -> then 4 AC
- +/- pembrolizumab if approved

R

L



T N M

Right Side

Left Side

Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:**Pathological Staging:**

Patient Name	اوزلي ابراهيم احمد	Case No.	340122
Sex/Age	Female 49 Y	Patient No.	24416
Registered Date	04/7/2026 03:44:50 PM	Referred By Dr.	Out Patient
Validation Date	08/7/2026 11:35:06 AM	Branch	Gustave Roussy
Reporting Date	08/7/2026 11:46:52 AM	Pathology No.	SR-778/26

Pathology report
(slide revision)

Clinical Data: Right breast cancer, conservative surgery and Axillary Lymph node dissection were done outside GRI-e

Nature of Specimen: Slide revision

Gross Examination:

-Eight FFPE blocks labelled (202296 and 200595-26) and 17 slides were received for slide revision

Microscopic examination:

- Histologic type: Invasive breast carcinoma, NST with medullary like features
- Histologic grade: 3 (differentiation 3, pleomorphism 3, mitosis 2)
- DCIS: Absent
- Microcalcification: Absent
- Angiolymphatic Invasion: Present
- Perineural Invasion: Absent
- Resection margins: Refer to previous report
- Tumor size: 1x0.5 cm (as described in previous report)
- Axillary Lymph nodes: Positive for tumor deposits in 10/15 Lns with extranodal extension
- Stage: pT1b N3

Review of IHC slides

- * IPT (All controls showed appropriate reactivity):
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 25%.

Conclusion:

Right breast cancer, Breast conservative surgery and axillary lymph node dissection (slide revision)

- Invasive breast carcinoma, NST with medullary-like features
- Resection margins: Free (as described in previous report)
- Axillary Lymph nodes: Positive for tumor deposits in 10/15 LNs with extranodal extension



طريقه رقم (14) - الدور الاول - مبنى رقم (1987) - برج الشمال - مصر الجديدة - القاهرة - مصر
 Room No. (14) - First Floor - Building No. 1987 - North Tower - El-Dokki - Cairo, Egypt

Patient Name	أززي إبراهيم أحمد	Case No.	340122
Sex/Age	Female 49 Y	Patient No.	24416
Registered Date	04/7/2026 03:44:50 PM	Referred By Dr.	Out Patient
Validation Date	08/7/2026 11:35:06 AM	Branch	Gustave Roussy
Reporting Date	08/7/2026 11:46:52 AM	Pathology No.	SR-778/26

IPT (slide revision)

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 25%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Prof. Dr. Maha Guinei


عنوان رقم (4) - (الطابق الأول) - مبنى رقم (967) - شارع النيل - مبنى رقم (967) - شارع النيل - مبنى رقم (967) - شارع النيل
 Room No. (4) - First Floor-Building No. 967-Nile Corniche - Mairie de Paris - Paris, France

Patient Name	ارزاق ابراهيم احمد	Case No.	358678
Sex/Age	Female 49 Y	Patient No.	25988
Registered Date	17/8/2026 02:05:19 PM	Referred By Dr.	Interventional Radiology
Validation Date	05/9/2026 11:00:47 AM	Branch	Gustave Roussy
Reporting Date	05/9/2026 11:20:56 AM	Pathology No.	2607/26

Pathology Report

Clinical Data: Right breast UOQ post-operative sequelae in a status post right breast conservative surgery for pathologically proven carcinoma.

Nature of Specimen: Tru-cut biopsy, collected on 17/08/2026, Cold ischemia time: 20 seconds.

Gross Examination:

Three grey, white tissue cores, ranging in length between 0.3 cm & 0.8 cm, totally submitted.

Microscopic Examination:

- Histopathological examination reveals cores of tissue showing fat necrosis & foreign body multinucleated giant cell reaction. No evidence of malignancy in section examined.

Diagnosis:

Right breast lesion (post-right breast conservative surgery status), Tru-cut biopsy:

- Consistent with resection site changes (fat necrosis & foreign body multinucleated giant cell reaction).
- No malignancy.

Signature

Prof. Dr. Gamal Abd El-Atty.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient's Name: Arzaq Ibrahim Ahmed Abdel Jawad
Date: 2026-08-22
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Neck Ultrasonography

multiple enlarged matted right supracalvicualr L.Ns seen showing irregular shape largest measuring 3cm with hypoechoic centers

The right and left lobes and isthmus of thyroid gland are of normal size, homogenous echo pattern, with regular outline.

Normal vascularity at CCDI.

No cystic or solid nodules detected.

The trachea is centralized.

Both carotid sheaths are free.

Conclusions

Right supraclavicular highly suspicious pathological L.Ns as described for further assessment

Dr.Sahl Mahmoud

Patient Name	ارفاق ابراهيم احمد	Case No.	358678
Sex/Age	Female 49 Y	Patient No.	25988
Registered Date	17/8/2026 02:05:19 PM	Referred By Dr.	Interventional Radiology
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Signature

Prof. Dr. Gamal Abd El-Atty.



خريطة رقم (4) - الدور الأول - مبنى الزئبق - مبنى التقييد - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qailema - Cairo, Egypt

تقرير طبي

السيد

تحية طيبة وبعد,

إيماء للطلب المقدم من

بشأن تقرير طبي للمريض

بقسم:

رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالة

yyyy-mm-dd --:-- --

تاريخ وساعة الخروج :

yyyy-mm-dd --:-- --

تاريخ وساعة الدخول :

وبعد الكشف والفحوصات والمتابعة وجد انه

wt 72 ht 165 sa 1.8 postmenop rt breast cancer undergone rt bcs on 24/5/2026 upfront outside gustave pt1 n3 10/15 idc with medullary feautres grade 3 triple -ve ki 25% initial pet ct rt supraclav ln suv 10 m 1.8 cm postop ,mri breast rt uoq birads 4 a non mass like enhancement for meoadj keytruda

تحريراً في ,

yyyy-mm-dd

التاريخ :

--:-- --

الوقت :

الطبيب المعالج :

يعتمد ,



A B S

Collection Date: 02/08/2026

Review Date: 29/08/2026

Referring Doctor: أ.د خالد عبد العزيز

الإستاذة/ أرواق إبراهيم أحمد

Patient No.: 203-25136

Accession No.: 203260823935

Age: 49Y

NEXT GENERATION SEQUENCING (NGS)

BRCA Report

Clinical Information

Arzak Ibraheem is a Cancer patient, tested for germline mutations in BRCA 1,2 for the possible applicability for targeted therapy.

Test Information

Extracted DNA using QIAamp DNA Mini kit from the patient's blood sample had successfully passed appropriate libraries according to the AmoyDx BRCA Pro Panel and sequenced on the MiSeq platform (Illumina). The minAmp (minimum depth) was not less than X200.

- The coding regions of BRCA1 & BRCA2 genes have been investigated using NGS technology.
- BRCA1 & BRCA2 genes have been implicated in hereditary breast, ovarian, prostate and pancreatic cancer predisposition syndrome.
- For patients with BRCA1/2 mutations (actionable pathogenic or Likely pathogenic Variants), The PARP inhibitors show clear benefit (Gradishar et al., 2021)
- All identified variants are evaluated with respect to their pathogenicity and causality, and were categorized into classes (1 – 5) as recommended by ACMG guidelines, where Class (1) Pathogenic, Class (2) Likely pathogenic, Class (3) Variant of Uncertain Significance (VUS), Class (4) Likely benign, Class (5) Benign (Richards S, et al., 2015).
- Our variant classification pipeline included variant interpreter by Illumina, Clinvar and Varsome (Kopanos C. et al., 2019).
- Only pathogenic and likely pathogenic variants with actionable therapeutic application are reported.
- The test results were reviewed, interpreted and reported by our scientific and medical experts.
- The classification of variants can change over the time.

Test result

No Actionable Pathogenic or Likely Pathogenic Variants Detected in BRCA1,2 genes.

No large rearrangement or Copy Number Variations (CNV) in BRCA1,2 genes.

Interpretation

- The patient is not harboring Pathogenic or Likely Pathogenic Variants Detected in BRCA1,2 genes.

- The patient is unlikely to benefit of PARPI

Recommendations

MLPA analysis of BRCA1 and BRCA2 genes is recommended to rule out the presence of large deletions/duplications or copy number variations.

We recommend Follow up with the referring Physician.

Please feel free to contact NGS team at Alfa laboratories (NGS@alfalabs.com).

Pt. Branch: السيد زئيب

Page 1 of 2

Printed by: أحمد حسن محمود 4396



L A B S

Collection Date: 02/08/2026

Review Date: 29/08/2026

Referring Doctor: أ.د خالد عبدالعزیز

Patient Name: الاستاذة/ ارزاق ابراهيم احمد

Patient No.: 203-25136

Accession No.: 203260823935

Age: 49Y

NEXT GENERATION SEQUENCING (NGS)

BRCA Report

Test limitations

1. Intronic variants/variants in untranslated region, promoters and intronic variants are not assessed using this method.
2. Variants not detected by the assay that was performed may impact the phenotype.

3. This test is using reference databases obtained from occidental communities.

4. Cases where the genetic variant is not identified may be due to limitations in current medical knowledge or testing technology.

5. Genetic tests are largely dependent on the clinical history, medical findings and family pedigree, and therefore clinical accurate interpretation of test results may require knowing the true biological relationships in a family. Misinterpretation of results may occur if the information provided is inaccurate or incomplete.

6. The interpretation of test results is done in accordance with the clinical history, family history and other medical and laboratory data.

7. Inaccurate results in genetic testing are very rare as these tests are highly specific and accurate.

References:

1. Garrett et al., BRCA1/BRCA2 :CanVIG-UK Gene-Specific Guidance 2026 (<https://www.cangene-canvaruk.org/canvig-uk-guidance>).2. Gradishar WJ, Moran MS, Abraham J, et al. NCCN Guidelines® Insights: Breast Cancer, Version 4.2021. J Natl Compr Canc Netw. 2021;19(5):484-493. Published 2021 May 1. doi:10.6004/jnccn.2021.0023
3. Kopanos C, Tsiolkas V, Kouris A, et al. VarSome: the human genomic variant search engine. Bioinformatics. 2019;35(11):1978-1980. doi:10.1093/bioinformatics/bty897
4. Richards S, Aziz N, Bale S, et al. Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology. Genet Med. 2015;17(5):405-424. doi:10.1038/gim.2015.30

Authorized and Reviewed by:

Dr. Samar Saad

Consultant, MD

Prof. Sahar Abd/Atty

Head of the NGS Unit



Pt Branch: السيد زئيب

Page 2 of 2

Reviewed By:

Printed by: احمد حسن محمود 4396

د. يوسف
روسل
أ. - ن



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

المسح الوطني
لصحة المرأة

09/08/2026

MDT FORM

Name:	أرزاق إبراهيم أحمد عواد	MRN:	11876
Age:	50 Y	ID:	27610202102802
Menopausal Sta	postm	Phone:*	01003239457
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		postop mri birads 4 non mass enhancement initial decision for clipping but biops	
Surgical History:*	bcs	ECOG Performance Status	☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4

Clinical Data:

R

L



T N M

BX:
 ER: PR: HER2: KI67:

History:

postmenop rt breast cancer undergone rt bcs on 24/5/2026 upfront
 outside gustave pt1 n3 10/15 idc with medullary features grade 3 triple -
 ve ki 25% initial pet ct rt supraday ln suv 10 m 1.8 cm
 postop ,mri breast rt uoq birads 4 a non mass like enhancement
 pt rec 2 weeks taxol carbopl
 initial decision for clipping of this birads4 lesionfor wider excision after
 ending pseudoneoa) tt
 true cut biopsy of rt breast (postop mri rt birads 4 non mass enhancement
) lesion 17/8/2026 consistent with resection site changes for remdt (is
 there need for clips or completion matectomy after that as biopsy is
 negative)

Sonomammography:

Clinical Staajng:

Biomarker/Molecular

Pathological Staajng:

Metis Workup:

Recommendations:

MDT Outcome & Plan

Continue systemic therapy
 for completion mastectomy post tit

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. حاتم عبد العزيز	د. محمد فوزي		

Patient Name	ارفاق ابراهيم احمد	Case No.	358678
Sex/Age	Female 49 Y	Patient No.	25988
Registered Date	17/8/2026 02:05:19 PM	Referred By Dr.	Interventional Radiology
Validation Date	05/9/2026 11:00:47 AM	Branch	Gustave Roussy
Reporting Date	05/9/2026 11:20:56 AM	Pathology No.	2607/26

Pathology Report

Clinical Data: Right breast UOQ post-operative sequelae in a status post right breast conservative surgery for pathologically proven carcinoma.

Nature of Specimen: Tru-cut biopsy, collected on 17/08/2026, Cold ischemia time: 20 seconds.

Gross Examination:

Three grey, white tissue cores, ranging in length between 0.3 cm & 0.8 cm, totally submitted.

Microscopic Examination:

- Histopathological examination reveals cores of tissue showing fat necrosis & foreign body multinucleated giant cell reaction. No evidence of malignancy in section examined.

Diagnosis:

Right breast lesion (post-right breast conservative surgery status), Tru-cut biopsy:

- Consistent with resection site changes (fat necrosis & foreign body multinucleated giant cell reaction).
- No malignancy.

Signature

Prof. Dr. Gamal Abd El-Atty.



خريطة رقم (4) - الدور الأول - مبنى القيد - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qailema - Cairo, Egypt

Patient's Name: Arzaq Ibrahim Ahmed Abdel Jawad
Date: 2026-08-22
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Neck Ultrasonography

multiple enlarged matted right supraclavicular L.Ns seen showing irregular shape largest measuring 3cm with hypoechoic centers

The right and left lobes and isthmus of thyroid gland are of normal size, homogenous echo pattern, with regular outline.

Normal vascularity at CCDI.

No cystic or solid nodules detected.

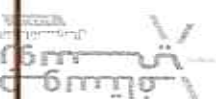
The trachea is centralized.

Both carotid sheaths are free.

Conclusions

Right supraclavicular highly suspicious pathological L.Ns as described for further assessment

Dr.Sahl Mahmoud



Gustave Roussy International-Egypt-Pathology Lab
مركز روستي الدولي للبحوث المرضية - مصر



Patient Name	الاناق الراحيم احمد	Case No.	340122
Sex/Age	Female 49 Y	Patient No.	24416
Registered Date	04/7/2026 03:44:50 PM	Referred By Dr.	Out Patient
Validation Date	08/7/2026 11:35:06 AM	Branch	Gustave Roussy
Reporting Date	03/10/2026 09:21:15 AM	Pathology No.	SR-778/26

Pathology report
(slide revision)

Clinical Data: Right breast cancer, conservative surgery and Axillary Lymph node dissection were done outside GRI-e

Nature of Specimen: Slide revision

Gross Examination:
-Eight FFPE blocks labelled (202296 and 200595-26) and 17 slides were received for slide revision

Microscopic examination:

- Histologic type: Invasive breast carcinoma, NST with medullary like features
- Histologic grade: 3 (differentiation 3, pleomorphism 3, mitosis 2)
- DCIS: Absent
- Microcalcification: Absent
- Angiolymphatic invasion: Present
- Perineural invasion: Absent
- Resection margins: Refer to previous report
- Tumor size: 1x0.5 cm (as described in previous report)
- Axillary Lymph nodes: Positive for tumor deposits in 10/15 Lns with extranodal extension
- Stage: pT1b N3

Review of IHC slides

- * IPT (All controls showed appropriate reactivity):
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 25%.

Conclusion:

- Right breast cancer, Breast conservative surgery and axillary lymph node dissection (slide revision)
- Invasive breast carcinoma, NST with medullary-like features
- Resection margins: Free (as described in previous report)
- Axillary Lymph nodes: Positive for tumor deposits in 10/15 LNs with extranodal extension



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Patient Name	ارزاق ابراهيم احمد	Case No.	340122
Sex/Age	Female 49 Y	Patient No.	24416
Registered Date	04/7/2026 03:44:50 PM	Referred By Dr.	Out Patient
Validation Date	08/7/2026 11:35:06 AM	Branch	Gustave Roussy
Reporting Date	03/10/2026 09:21:15 AM	Pathology No.	SR-778/26

IPT (slide revision)

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 25%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Maha Guimei



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